

The Global Opioid Crisis: Lessons from the United States of America and Beyond

Adam Milad and Mohamed Milad* 

The opioid epidemic, initially perceived as a predominantly American crisis, has rapidly evolved into a global public health emergency with devastating consequences extending far beyond U.S. borders.^{1,2} Opioid misuse and overdose deaths have escalated internationally, driven by widespread availability, prescription overuse, illicit trafficking, and the emergence of potent synthetic opioids such as fentanyl.^{3,4} The recent article by Ibteisam Madhi, published in this issue of the Journal of the Best Available Evidence in Medicine, offers timely reflections on local responses to the crisis in San Joaquin County, California—responses that echo broader global challenges.⁵

Madhi's qualitative investigation, grounded in interviews with stakeholders across healthcare, law enforcement, and public health, highlights the multifaceted nature of the opioid problem.⁵ Initiatives led by the San Joaquin County Opioid Safety Coalition (OSC)—including public education, naloxone distribution, and prescriber training—represent important steps forward. Nonetheless, persistent barriers such as stigma, limited funding, and treatment access continue to hinder progress, as they do in many parts of the world.⁵

While the paper provides valuable contextual insight, future work might benefit from incorporating complementary quantitative data. Specific figures regarding program uptake, intervention reach, and outcome trends would strengthen the evidence base and support clearer policy translation.⁵ However, the article contributes meaningfully to our understanding of the complexities involved in responding to this evolving crisis.

Internationally, similar challenges are frequently encountered, particularly in countries with under-resourced health systems and fragmented public health strategies.¹ The spread of opioid-related harms in Canada, Australia, parts of Europe, and across Asia and Africa reflects shared vulnerabilities that call for coordinated global responses.^{1,6}

In Australia, the 2022–2023 AIHW National Drug Strategy report indicated that 2.2% of Australians used prescription opioid for illicit and non-medical purposes. Opioid-related deaths in 2002 were 375 and 926 in 2022, an increase of 147%.⁷ The report indicated that almost two-thirds of unintentional drug-induced deaths involving opioids occurred among people aged 40 and over in 2022, and that unintentional drug-induced deaths exceeded road traffic fatalities in the same year, highlighting the increasingly serious public health issue of opioid-related deaths.⁷

As the crisis continues to unfold, it is increasingly clear that no single approach will suffice. Local experiences such as those described in San Joaquin County provide an important lens through which to consider global solutions—ones grounded in collaboration, knowledge sharing, and a commitment to both evidence and compassion.^{1–5}

Disclosure Statement

The authors declare no conflicts of interest.

Invited: 20/03/2025

 OPEN ACCESS

Accepted: 02/04/2025

Published online: 26/04/2025

Psychiatric Services, Queensland Health, Brisbane, Australia

*Corresponding Author:

mohamed.milad@health.qld.gov.au

